Daily Life and Wellness: How Hispanic Cultural Origins Shape Health Behaviors

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Abstract

This literature review synthesizes ten peer-reviewed empirical studies (2025) examining how cultural factors shape health behaviors among Hispanic/Latino populations in the United States across smoking cessation, physical activity, diabetes prevention, dementia caregiving, obesity, mental health, and substance use. Cultural factors operate at multiple levels: language preference affects healthcare navigation; familismo functions dually as support and stressor; acculturation paradoxically increases discrimination exposure among US-born individuals; health literacy mediates preventative behaviors (62% variance explained); and discrimination impacts mental health outcomes. Spanish-language, culturally adapted interventions achieved high retention rates (90%-100%). Effective interventions must recognize within-group heterogeneity, prioritize health literacy, and address structural barriers.

Key words: Hispanic health, acculturation, health literacy, health disparities, cultural adaptation, preventative behaviors, mental health

Introduction

The Hispanic/Latino population in the United States represents a diverse group with varying cultural backgrounds, migration histories, and health outcomes. Understanding how cultural origins shape health behaviors is essential for developing effective, culturally tailored interventions that address health disparities in this growing population. While much research has documented health disparities affecting Hispanic communities, less attention has been paid to the specific ways in which cultural factors, such as familismo (family orientation), acculturation levels, language preferences, health literacy, and ethnic identity, influence daily health behaviors and wellness practices.

This literature review examines recent empirical research (2025) investigating the relationship between Hispanic cultural origins and health behaviors across multiple studies. The ten studies reviewed included participants from diverse Hispanic subgroups: Mexican and Mexican American (the largest represented group), Puerto Rican, Cuban, Dominican, Central American (including Ecuadorian), and South American populations. Participants were predominantly first and second-generation immigrants, though samples also included third-generation and U.S.-born individuals without recent immigration history. Geographic settings spanned major U.S. urban areas including New York City, Los Angeles, Chicago, and communities across California, Texas, Washington, Oregon, Utah, and national online samples.

The health behaviors and outcomes examined include smoking cessation and tobacco use, physical activity and sedentary behavior, diabetes prevention and management, obesity and weight management, dementia caregiving and healthcare navigation, traumatic brain injury recovery, mental health outcomes (anxiety, depression, loneliness), discrimination experiences, and substance use (alcohol, marijuana, hard drugs) among youth. Cultural factors analyzed across these domains include familismo and family support dynamics, language preference and English proficiency, acculturation level and acculturative stress, health literacy and educational attainment, cultural beliefs about personal responsibility, and experiences of discrimination and microaggressions.

The review synthesizes findings from the ten peer-reviewed empirical studies to identify common cultural themes and their implications for health interventions. Table 1 provides an overview of the included studies.

Overview of Included Studies					
Study	**Design**	**Sample Size**	**Population**	**Health Domain**	**Key Findings**

Overview of Included Studies					
Correa-Fernandez et al. (2025)	Qualitative (SEM)	18	Spanish speaking Latinx adults with Psychological distress	Smoking cessation	Familismo operates dually: family support helpful but family demands create stress
Estrada et al. (2025)	Qualitative Interviews	19	Latino dementia caregivers, NYC	Dementia caregiving	Caregivers experience isolation when family doesn't share responsibilities contrary to cultural expectations
Garcia et al. (2025)	Qualitative (CFIR)	12	Hispanic TBI program implementers	Traumatic brain injury navigation	Bilingual/bicultural navigators essential; immigration concerns major barrier
Gonzalez et al. (2025)	Qualitative interviews	15	Hispanic men with prediabetes	Diabetes prevention	Surprise at diabetes risk despite family history; work environment and cultural foods create barriers
Loya et al. (2025)	Pre-post pilot intervention	21 (19 completed)	Hispanic adults with Type 2 Diabetes	Physical activity	Steps nearly doubled (23,006 → 43,001, p=.005); 90.5% retention during COVID-19
Odame et al. (2025)	Cross-sectional national survey	1,126	Hispanic/Latino adults (55.51% US-born)	Mental health & discrimination	38.19% experienced daily/weekly discrimination; US-born 30% more likely than foreign born; anxiety/depression increases risk 138%
Sanchez-Johnsen et al. (2025)	CBPR baseline	199	Latina women (Mexican, Puerto Rican)	Obesity	Significant within-group differences: Mexican women younger, less educated, higher Spanish preference, lower obesity than Puerto Rican women
Tailakh et al. (2025)	Cross-sectional path analysis	149	Hispanic adults, Southern California	Health literacy & preventative behaviors	Healthy literacy explained 62% variance; education strongest predictor (β = .74); health literacy → adherence (β=.51)
Villamil Grest et al. (2025)	Longitudinal LCA (3 timepoints, 2005 - 2016)	1,179	Latino/Hispanic youth. Los Angeles	Substance use & ACEs	4 adversity classes identified: "Psychological abuse & discrimination" class showed higher alcohol use (B=.316, p=.04)
Zlatar et al. (2025)	Pre-post pilot intervention	10	Hispanic older adults (mean age 62.7)	Physical activity	100% completion; high satisfaction (4.6/5); barriers: low self-efficacy, time; facilitators: social support, Fitbit

Source: Reviewed literature (2025).

Figure 1. How Hispanic Cultural Origins Shape Health Behaviors

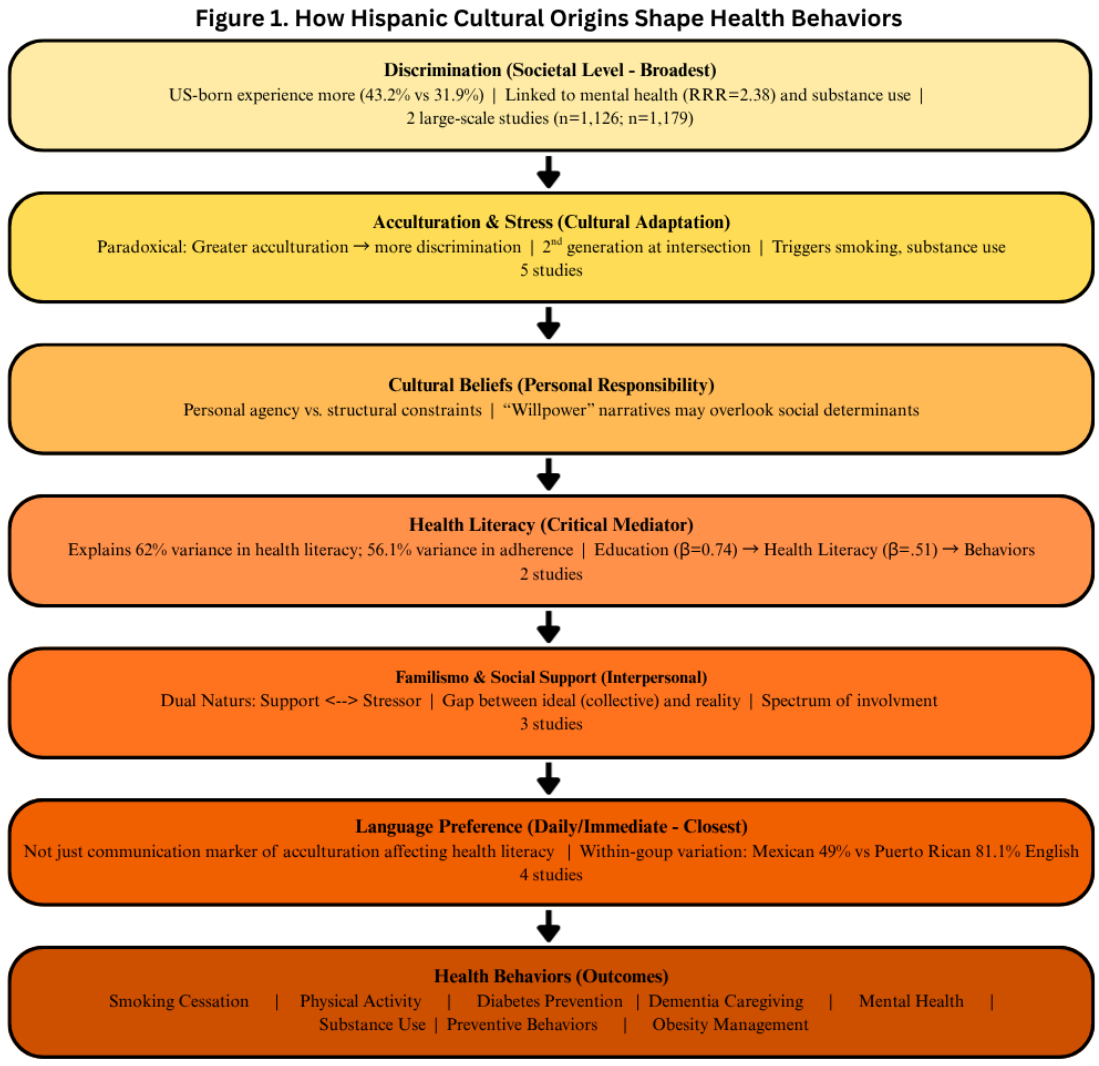

Figure 1. Conceptual Model: How Hispanic Cultural Origins Shape Health Behaviors.

Multilevel framework illustrating how cultural factors operate at six levels, from language preference (most immediate) to discrimination (broadest societal) to influence health behaviors. Health literacy (middle level, highlighted) emerges as critical mediator explaining 62% of variance in preventive behaviors. This model synthesizes findings from 10 empirical studies examining smoking cessation, physical activity, diabetes prevention, caregiving, obesity, mental health, substance use, and preventive behaviors among Hispanic/Latino populations. Source: cnceptual synthesis of reviewed studies.

Methodology

This review includes ten peer-reviewed empirical studies published in 2025 that examined Hispanic/Latino health behaviors through cultural lenses. Studies were identified through searches in ProQuest, PsychINFO, Embase databases conducted in January 2026. Search terms included a combination of: ("Hispanic" OR "Latino" OR "Latina" OR "Latinx" OR "Mexican American" OR "Puerto Rican") AND ("health behavior" OR "health outcome" OR "preventive behavior") AND ("cultural factor" OR "familismo" OR "acculturation" OR "health literacy" OR "language preference") AND ("2025"[Publication Date]), Additional studies were identified through citation tracking of relevant articles.

Studies were selected based on: (1) peer-reviewed journal articles with original empirical data, (2) focus on Hispanic/Latino population in the United States, (3) examination of cultural factors influencing health behaviors, (4) published in 2025, (5) empirical research designs (qualitative, quantitative, and mixed methods). Excluded were literature reviews (n=3), and theoretical papers without empirical data (n=2), and conference presentations not yet peer-reviewed (n=1). From my original search of studies that were taken into consideration for this paper.

The ten studies had diverse methodological approaches, four qualitative studies (Correa-Fernández et al., Estrada et al., Garcia et al., Gonzalez et al.), three quantitative cross-sectional studies (Sánchez-Johnsen et al., Tailakh et al., Odame et al.), one longitudinal study (Villamil Grest et al.), and two pilot intervention studies (Loya et al., Zlatar et al.). Sample sizes ranged from 10 to 1,179 participants.

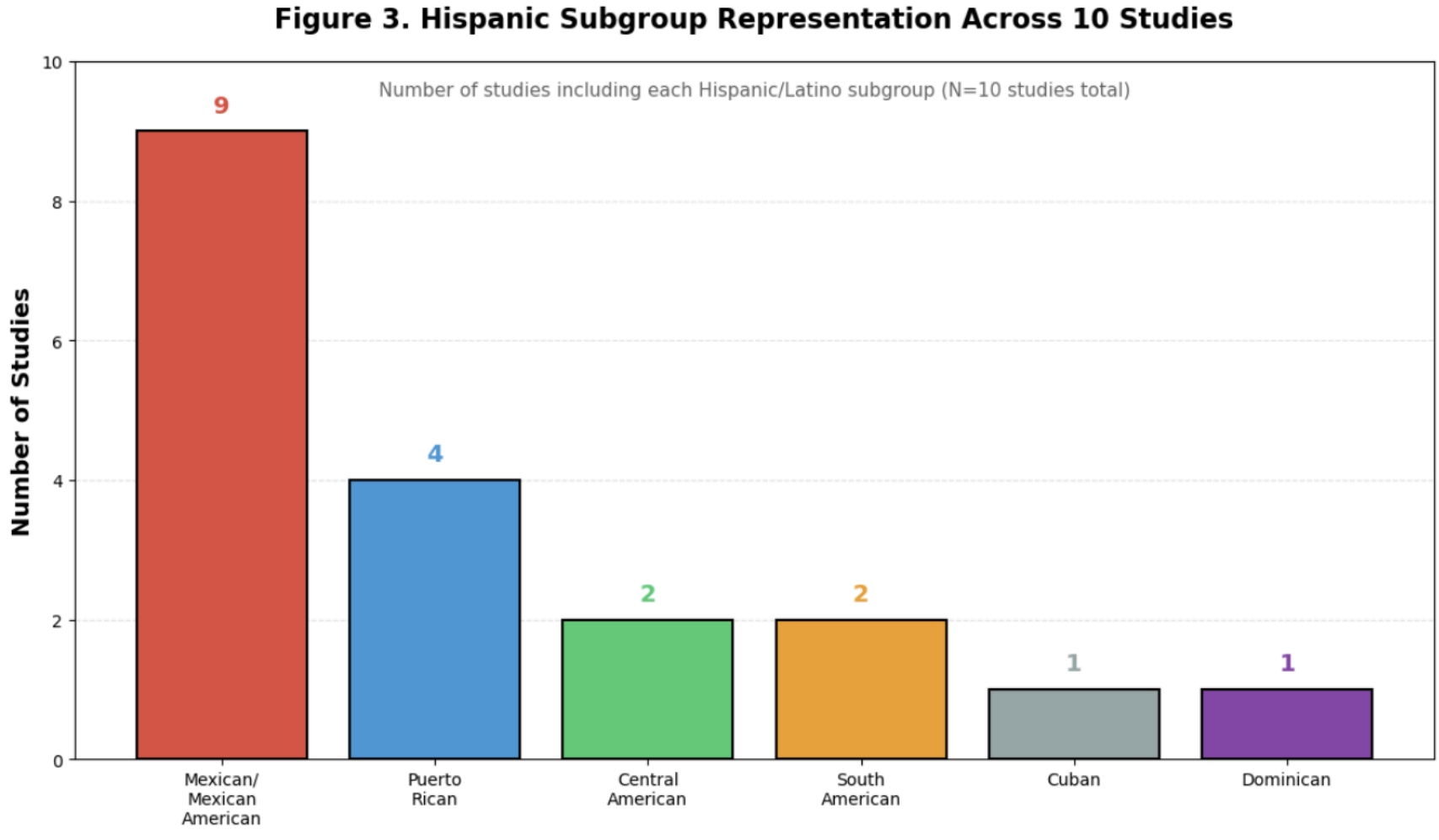

Figure 3. Hispanic Subgroup Representation Across 10 Studies. Bar chart showing number of studies including each Hispanic/Latino subgroup. Mexican/Mexican American represented in 9 of 10 studies; Cuban, Dominican, Central and South American groups critically underrepresented (1-2 studies each). Source: analysis of reviewed studies.

Figure 3 illustrates the distribution of Hispanic subgroup representation across the ten studies, revealing significant overrepresentation of Mexican/Mexican American populations (9 of 10 studies) and critical underrepresentation of Cuban, Dominican, Central American, and South American groups (1-2 studies each). This pattern reflects a broader limitation in Hispanic health research and constrains the generalizability of findings presented in this review.

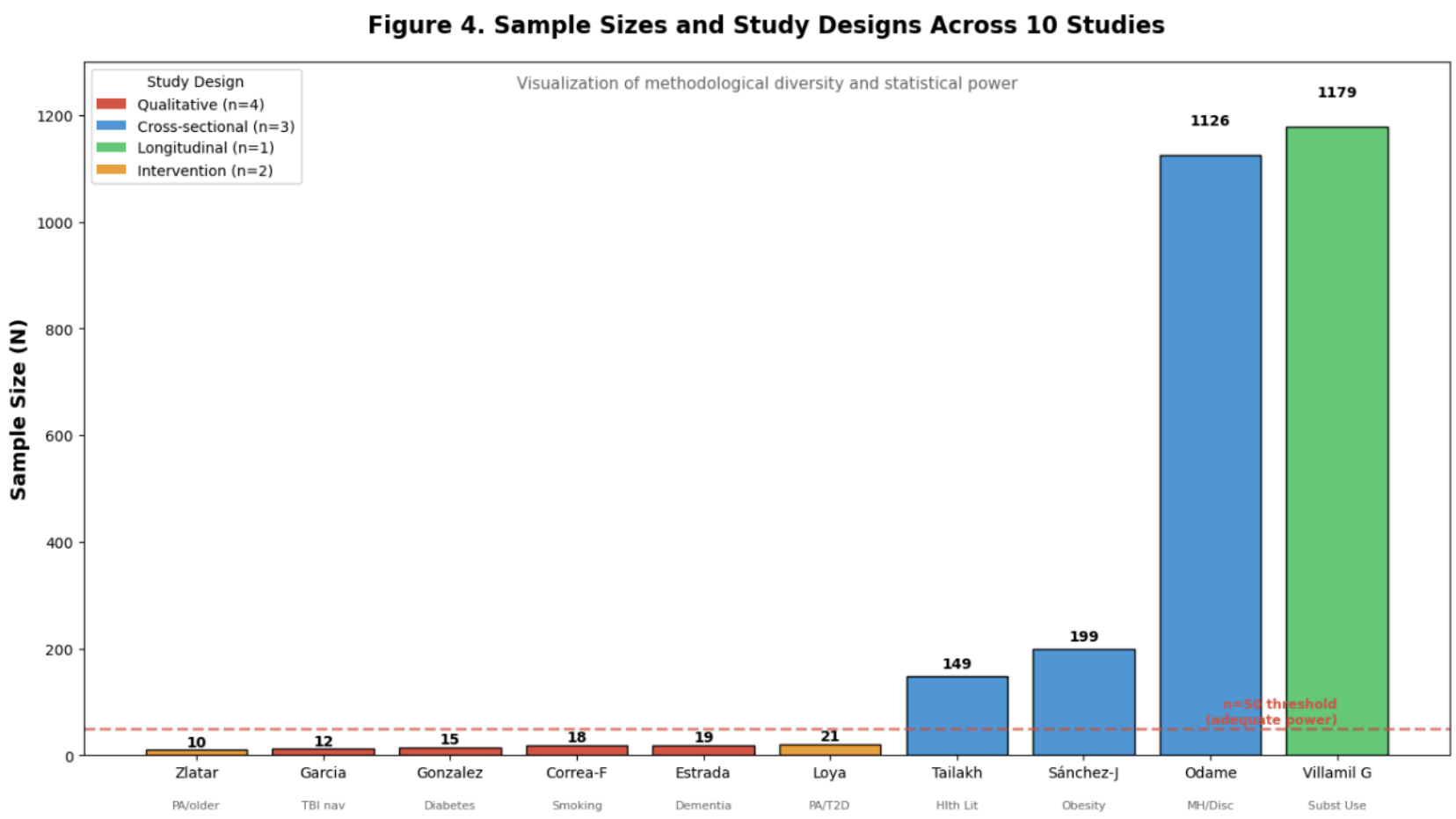

Figure 4 illustrates this methodological diversity and the substantial variation in statistical power across studies.

Figure 4. Sample Sizes and Study Designs Across 10 Studies. Visualization of methodological diversity and statistical power across the reviewed studies. Sample sizes range from n=10 (Zlatar et al.'s pilot physical activity intervention) to n=1,179 (Villamil Grest et al.'s longitudinal study), a 118-fold difference. Studies are color-coded by design: qualitative studies (red, n=4) range from 12-19 participants; cross-sectional quantitative studies (blue, n=3) range from 149-1,126; the sole longitudinal study (green) had 1,179 participants; and pilot intervention studies (orange, n=2) had 10-21 participants. The red dashed line indicates n=50, a common threshold for adequate statistical power to detect medium effect sizes. Six of ten studies (60%) fall below this threshold, limiting their statistical power. Only two large-scale studies (Odame, Villamil Grest) with n>1,000 provide robust quantitative evidence for population-level patterns. Source: analysis of reviewed studies.

The Role of Cultural Factors in Health Behaviors: Cross-Cutting Themes

The following sections examine cultural factors that emerged across multiple studies, organized from the most immediate, daily experiences (language use) outward through increasingly broader contexts (family, cultural adaptation, individual capacities, and societal forces). This framework shows how intimate daily interactions expand to encompass family dynamics, cultural identity process, individual knowledge and beliefs, and societal-level discrimination.

Language Preference and Healthcare Access

Language emerged as the most immediate cultural influencing health behaviors and healthcare navigation across four studies. Garcia et al. (2025) found that bilingual and bicultural patient navigators were essential facilitators in a traumatic brain injury program for Hispanic children, with 66.7% of implementation personnel Spanish-speaking and able to provide culturally relevant education materials in families' preferred language. Significantly, families' immigration concerns created hesitancy to seek assistance, suggesting language barriers intersect with broader structural vulnerabilities.

Language preferences varied substantially by Hispanic subgroups, challenging assumptions of linguistic homogeneity. Sánchez-Johnsen et al. (2025) reported that Mexican women showed significantly higher Spanish preference than Puerto Rican women (49% English preference vs 81.1% Puerto Rican, $p<.001$). This finding demonstrates that health interventions cannot assume language needs based solely on Hispanic ethnicity and must assess individual and subgroup preferences.

Correa-Fernández et al. (2025) specifically recruited Spanish-preferring smokers and documented strong preferences for cessation interventions delivered in Spanish with culturally relevant elements. Among their sample of 18 participants, 73.7% preferred Spanish for daily communication, highlighting the continued importance of Spanish-language health services even among U.S. residents. Tailakh et al. (2025) revealed the connection between language and health literacy, finding that speaking English at home was a significant predictor of higher health literacy ($\beta=.25$, $p<.05$) among 149 Hispanic adults. This association suggests that language preference operates not only as a communication issue but as a marker for broader acculturation processes.

Study	**N**	**Population**	**Key Language Findings**	**Implication**
Garcia et al. (2025)	12	TBI program implementers	66.7% Spanish-speaking navigators essential; immigration concerns create hesitancy	Bilingual navigators are facilitators; language intersects with structural vulnerabilities
Sánchez-Johnsen et al. (2025)	199	Latina women (Mexican, Puerto Rican)	Mexican women: 49% English preference; Puerto Rican: 81.1% English preference ($p<.001$)	Cannot assume language homogeneity within Hispanic populations
Correa-Fernández et al. (2025)	18	Spanish-preferring smokers	73.7% preferred Spanish for daily communication; strong preference for Spanish interventions	Spanish-language services remain critical for U.S. residents
Tailakh et al. (2025)	149	Hispanic adults	Speaking English at home predicted higher health literacy ($\beta=.25$, $p<..05$)	Language is marker of acculturation affecting health knowledge

Source: Garcia et al. (2025), Sánchez-Johnsen et al. (2025), Correa-Fernández et al. (2025), and Tailakh et al. (2025).

Familismo and Social Support

Expanding from individual language use to family relationships, familismo emerged across three qualitative studies as operating dually rather than uniformly. Correa-Fernández et al. (2025) found that while family support could facilitate smoking cessation, family demands simultaneously created stress that triggered smoking behavior among Spanish-speaking Latinx adults (n=18). Participants described a spectrum of desired family involvement in cessation efforts, suggesting that interventions should be flexible rather than prescriptive about family participation.

Estrada et al. (2025) identified lack of family support as a key challenge faced by Latino dementia caregivers (n-19) in New York City. Many caregivers experienced profound isolation when other family members did not understand dementia or share caregiving responsibilities, directly contradicting cultural expectations of collective family caregiving. This gap between cultural ideals

(familismo as shared responsibility) and lived reality (isolated primary caregivers) created additional stress and disappointment for caregivers who had anticipated more family involvement.

Zlatar et al. (2025) found that family relationships and social support were important considerations in culturally adapting a physical activity intervention for older Latino adults (n=10). The community advisory board emphasized the importance of family involvement and social connection in promoting sustained physical activity, through the intervention ultimately focused on individual participants rather than family units.

Across all three studies, the consistent finding was that familismo operates complexly, neither universally positive nor negative, requiring interventions to assess rather than assume family dynamics and desired involvement levels.

Study	**N**	**Health Domain**	**Familismo as Support**	**Familismo as Stressor**	**Key**
Correa-Fernández et al. (2025)	18	Smoking cessation	Family support helpful for quitting	Family demands create stress the triggers smoking	Dual role; spectrum of desired involvement
Estrada et al. (2025)	19	Dementia caregiving	Cultural expectation of collective caregiving	Isolation when family doesn't share responsibility; gap between ideal and reality	Lack of support contradicts cultural expectations
Zlatar et al. (2025)	10	Physical activity	Family relationships important for sustain PA; social connection emphasized by CAB	Intervention focused on individuals, not family units	Family involvement valued but implementation varies

Source: Correa-Fernández et al. (2025), Estrada et al. (2025), and Zlatar et al. (2025).

Acculturation and Acculturative Stress

Moving outward from family to broader cultural identity processes, acculturation as both protective and risk factor across multiple studies. Correa-Fernández et al. (2025) identified acculturation stress from work demands, financial pressures, and cultural adaptation challenges as major triggers for smoking behavior among Spanish-speaking Latinx adults (n=18). The stress of navigating between cultural contexts created psychological distress that participants described as directly precipitating smoking episodes.

The relationship between acculturation and health outcomes varies by generation and birthplace. Odame et al.'s (2025) finding that US-born Hispanic/Latino individuals experienced more discrimination than foreign-born counterparts (43.20% vs 31.94%, $p<.001$) suggests that greater acculturation may paradoxically increase exposure to discrimination. This pattern also appeared in Villamil Grest et al.'s (2025) sample, where 73.88% were second-generation (born in US with foreign-born parent(s)), positioned at the intersection of two cultures and potentially experiencing heightened acculturative stress.

Both Loya et al. (2025) and Zlatar et al. (2025) incorporated acculturation measures in their physical activity interventions, recognizing that acculturation level influences intervention engagement. Loya et al. found participants had notably low acculturation (mean SASH score 1.53 on 1-5 scale, indicating strong Hispanic identification), with all participants speaking Spanish as their primary home language. This cultural context shaped intervention design, with materials delivered entirely in Spanish at a 5th grade reading level.

Study	N	Design	Acculturation Measure/Indicator	Key Finding	Protective or Risk
Correa-Fernández et al. (2025)	18	Qualitative	Acculturative stress (work, finances, adaptation)	Triggers smoking behavior	Risk
Odame et al. (2025)	1,126	Cross-sectional	Birthplace (US-born vs. foreign-born)	US-born 30% more likely to experience discrimination ($p<.001$)	Risk (paradoxical)
Villamil Grest et al. (2025)	1,179	Longitudinal	Generation status (73.88% second generation)	Second generation at intersection of two cultures; heightened stress	Risk
Loya et al. (2025)	21	Intervention	SASH score (mean 1.53/5.0 = very Hispanic)	Low acculturation; 100% Spanish at home shaped intervention design	Context for intervention
Zlatar et al. (2025)	10	Intervention	Language (90% Spanish-speaking)	Low acculturation context; culturally adapted intervention	Context for intervention

Source: Correa-Fernández et al. (2025), Odame et al. (2025), Villamil Grest et al. (2025), Loya et al. (2025), and Zlatar et al. (2025).

Health Literacy as a Critical Mediator

At the level of individual knowledge and capacity, health literacy emerged as a crucial mediator between cultural factors and health behaviors. Tailakh et al. (2025) conducted path analysis demonstrating that their model explained 62% of variance in health literacy and 56.1% of variance in adherence to preventive behaviors among 149 Hispanic adults. Higher education was the strongest predictor of health literacy ($\beta=.74$), which in turn strongly predicted adherence to preventive health behaviors ($\beta=.51$). These findings establish health literacy as a key individual-level mechanism through which broader structural factors (education access) and cultural factors (language, as noted earlier) influence health outcomes.

Educational gaps that constrain health literacy were evident in Gonzalez et al.'s (2025) study, where 33.3% of Hispanic men with prediabetes had not completed high school. These men expressed surprise at their diabetes risk despite family history, suggesting knowledge gaps that health literacy interventions could address. The disconnect between family health history (a known risk factor) and personal risk perception indicates that health literacy deficits prevent individuals from translating family health patterns into preventive action

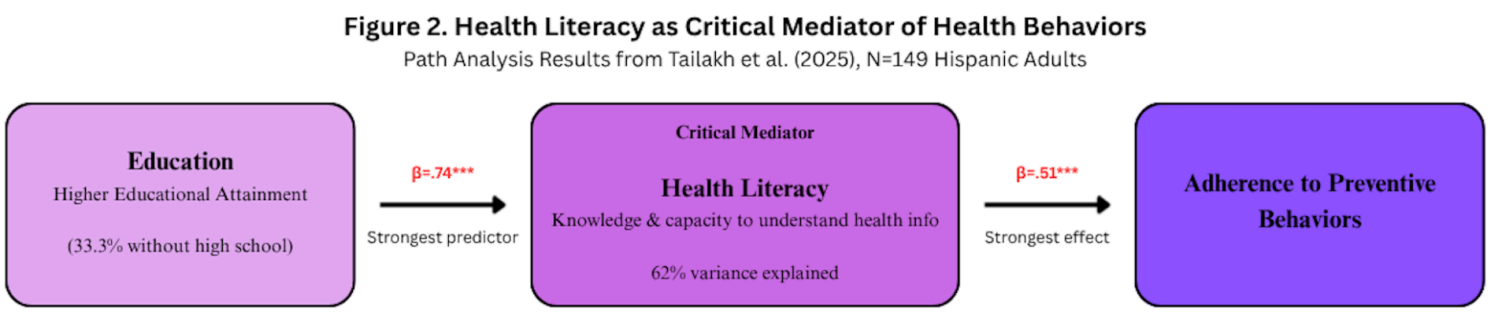

Figure 2. Health Literacy as Critical Mediator of Health Behaviors. Path analysis results from Tailakh et al. (2025) demonstrate that health literacy operates as a critical mediator between education and adherence to preventive behaviors among 149 Hispanic adults in Southern California. Education is the strongest predictor of health literacy ($\beta=.74$, $p<.001$), explaining 62% of variance in health literacy. Health literacy, in turn, strongly predicts adherence to preventive health behaviors ($\beta=.51$, $p<.001$), explaining 56.1% of variance in adherence. Source: Adapted from Tailakh et al. (2025).

Study	N	Design	Health Literacy Measure	Key Findings	Mediating Pathway
Tailakh et al. (2025)	149	Cross-sectional path analysis	Path analysis model	62% variance in health literacy explained; 56.1% variance in adherence explained)	Education → Health literacy ($\beta=.74$) → Adherence ($\beta=.51$)

Gonzalez et al. (2025)	15	Qualitative	Educational attainment (33.3% < high school)	Surprise at prediabetes risk despite family history; knowledge gaps)	Health literacy deficits prevent translating family history into preventative action

Source: Synthesis of Tailakh et al. (2025) and Gonzalez et al. (2025).

Cultural Beliefs About Personal Responsibility

Closely related to health literacy, cultural beliefs about personal responsibility emerged as shaping health behaviors and treatment-seeking. Gonzalez et al. (2025) found that Hispanic men with prediabetes (n=15) viewed lifestyle change as manageable and emphasized personal agency as necessary for change, yet simultaneously noted that restrictive work environments, overwhelming schedules, and carbohydrate-rich cultural food preferences created substantial barriers. This tension between beliefs about individual control and recognition of structural constraints reflects a complex understanding of health behavior determinants.

Similarly, Correa-Fernández et al. (2025) found that Spanish-speaking Latinx smokers strongly endorsed beliefs about personal responsibility and "willpower" as necessary for quitting, even while acknowledging psychological distress, addiction, and social/environmental triggers for smoking. This emphasis on personal agency may reflect cultural values of self-determination (personalismo) but could indicate internalization of individual-responsibility narratives that overlook structural determinations of health.

Discrimination and Mental Health Disparities

Finally, at the broadest societal level, discrimination emerged across two large-scale quantitative studies as a fundamental stressor affecting Hispanic/Latino mental health and substance use. Odame et al. (2025) examined discrimination experiences among 1,126 Hispanic/Latino adults during COVID-19, finding that 38.19% experienced discrimination daily or weekly. Discrimination was strongly associated with mental health symptoms: participants with anxiety or depression showed 138% higher risk of experiencing daily/weekly discrimination (RRR=2.38, 95% CL 1.54-3.70), while each unit increase in loneliness increased discrimination risk by 29% (RRR=1.29, 95% Cl 1.17-1.42).When stratified by birthplace, anxiety/depression predicted discrimination only among US-born individuals (269% higher risk, RRR=3;69), while loneliness predicted discrimination for both groups.

Villamil Grest et al. (2025) examined how discrimination combines with other adversities to affect substance use among 1,179 Latino youth followed longitudinally. Using latent class analysis, they identified four patterns of adverse experiences, with the "Psychological abuse & discrimination" class (31% of sample) showing significantly higher alcohol use compared to those experiencing mainly microaggressions (B=.316, p=.04). Youth in the Psychological abuse & discrimination' class showed significantly higher alcohol use than the 'Mainly Microaggressions' class'.

Study	**N**	**Design**	**Discrimination**	**Key Finding**	**Nativity/Generation Differences**
Odame et al. (2025)	1,126	Cross-sectional national survey	Daily/weekly discrimination frequency	38.19% experienced discrimination daily/weekly; anxiety/depression RRR-2.38; loneliness RRR=1.29	US-born: 43.20%; Foreign-born:31.94% (p<0.001). Anxiety/depression predicted discrimination only for US-born (RRR=3.69)
Villamil Grest et al. (2025)	1,179	Longitudinal LCA (3 timepoints)	Latent class analysis of adverse experiences	"Psychological abuse & discrimination" class (31%) showed higher alcohol use (B=.316, p=0.04)	73.88% second generation (at intersection of cultures)

Source: Synthesis of Odame et al. (2025) and Villamil Grest et al. (2025).

Together, these sections illustrate how cultural factors operate at multiple levels, from the immediate experiences of languages use in daily life, through family relationships and cultural identity navigation, to individual knowledge and beliefs, and ultimately to societal discrimination that shapes health outcomes across the lifespan.

Key Findings in Health Behavior Domains

While the previous section examined cultural factors that cut across health domains, this section explores how these factors manifest in specific health behaviors. Studies are organized from individual-level behavioral interventions (physical activity, smoking cessation) through family-mediated health management (caregiving, diabetes prevention) to population-level health outcomes (obesity patterns, mental health, substance use).

Physical Activity Interventions: Comparing Two Culturally Adapted Approaches

Two pilot intervention studies examined physical activity promotion among Hispanic adults with chronic conditions, both achieving exceptional retention and feasibility despite different populations and approaches. Loya et al. (2025) tested "Salud Paso por Paso" among 21 adults with Type 2 Diabetes (90% female, 95% Mexican, mean age 53), while Zlatar et al. (2025) culturally adapted "De Pie y a Movernos" for 10 older adults (mean age 62.7, 90% female, 90% Spanish-speaking, 90% Mexican/Mexican American).

Both studies shared key design elements: community-engaged development processes, Spanish-language delivery, culturally tailored content, and pre-post designs measuring feasibility and preliminary efficacy. Both achieved remarkable retention (Loya: 90.5%; Zlatar: 100%), far exceeding typical physical activity intervention retention rates and suggesting that cultural adaptation enhances engagement.

However, the studies differed in several important ways. Loya et al.'s 6-week intervention was longer, group-based, and demonstrated significant physical activity increases: moderate PA hours increased 3.16 hours per week ($p<.001$), steps nearly doubled from 23,006 to 43,001 ($p=.005$); and weekly PA minutes more than doubled from 106 to 224 ($p<.001$). Participants exceeded AHA's recommended 150 minutes/week by intervention end. Interestingly, participants showed high baseline self-efficacy (mean 3.53/4.0) that remained stable, suggesting they possessed confidence but lacked structured opportunity and social support, which is the intervention provided.

Zlatar et al.'s 3-week intervention was shorter, entirely remote/individualized, and focused on dementia prevention messaging. While quantitative PA changes were not reported, qualitative findings revealed barriers (lack of awareness, low self-efficacy, time constraints, health conditions, weather) and facilitators (awareness of cognitive benefits, accountability, social support, Fitbit use, habit formation). Satisfaction was high (4.6/5), and participants requested additions including group activities and more staff interaction, suggesting that combining Zlatar's remote approach with Loya's group format might be optimal.

Both studies demonstrated that low-acculturated, Spanish-speaking Hispanic adults successfully engage with culturally adapted PA intervention. Loya participants had very low acculturation (SASH=1.53/5.0) with 100% speaking Spanish at home; Zlatar participants were 90% Spanish-speaking. This consistency suggests that Spanish-language, culturally grounded interventions are essential, not optional, for reaching this population.

Feature	Loya et al. (2025)	Zlatar et al. (2025)	Comparison
Population	21 adults with Type 2 Diabetes	10 older adults (mean age 62.7)	Different chronic conditions and age groups
Demographics	90% female, 95% Mexican, mean age 53	90% female, 90% Spanish-speaking, 90% Mexican/Mexican American	Both predominantly female, Mexican/Mexican American, Spanish-speaking
Acculturation	Very low (SASH=1.53/5.0); 100% Spanish at home	90% Spanish-speaking	Both low-acculturated populations
Duration	6 weeks	3 weeks	Loya longer
Format	Group-based	Remote/individualized	Different delivery models
Language	Spanish (5th grade reading level)	Spanish	Both Spanish-language
Development	Community-engaged	Community advisory board (Barrera & Castro framework)	Both culturally adapted through community input
Retention	90.5% (19/21)	100% (10/10)	Both exceptional; far exceed typical PA interventions
Primary Outcomes	Steps doubled (23,006 → 43,001, p=.005); PA minutes doubled (106 → 224, p<.001)	Qualitative barriers/facilitators; Satisfaction 4.6/5	Loya quantitative PA changes Zlatar feasibility/acceptabillity
Self-efficacy	High at baseline (3.53/4.0), remained stable	Identified low self-efficacy as barrier	Loya participants had confidence but lacked opportunity
Key Findings	Participants had confidence but needed structured opportunity and social support	Participants requested more group activities and staff interaction	Both suggest combining remote accessibility with group/social elements

Source: Analysis of Loya et al. (2025) and Zlatar et al. (2025).

Smoking Cessation and Psychological Distress

Correa-Fernández et al.. (2025) examined smoking among 18 Spanish-preferring Latinx adults with psychological distress (mean age 39.6, 57.9%, women, 73.6% foreign-born). Using the Social Ecological model, they identified psychological distress from anxiety, work stress, financial stress, and acculturation as major drivers of smoking. At the organizational level, participants reported little prior cessation treatment experience but expressed preferences for combination individual/group formats, in-person delivery, Spanish language interactive and personalized approaches, and incorporation of cultural elements like music and art. Participants described a spectrum of desired family involvement, suggesting interventions should be flexible rather than prescriptive about family participation, a finding that parallels the physical activity studies' emphasis on individual approaches within culturally grounded frameworks.

Family-Mediated Health Management: Caregiving and Diabetes Prevention

Two qualitative studies examined how family dynamics shape health management behaviors, both revealing tensions between cultural expectations and lived realities. Estrada et al. (2025) studied 19 Latino dementia caregivers in New York City, while Gonzalez et al. (2025) interviewed 15 Hispanic men with prediabetes (60% under age 50, 66.7% Spanish-preference) in East Harlem.

Both studies identified gaps between cultural ideals and actual support. Estrada et al. found caregivers experienced profound isolation when other family members didn't understand dementia or share responsibilities, contradicting cultural expectations of collective family caregiving. This gap between familismo ideals (shared responsibility) and reality (isolated primary caregivers) created additional stress. Similarly, Gonzalez et al. found men emphasized personal agency for lifestyle change yet simultaneously noted that

work environment and cultural food preferences (carbohydrate-rich staples like rice, beans, tortillas) created substantial barriers revealing tension between individual responsibility beliefs and structural constraints.

Both studies also revealed knowledge gaps: Estrada's caregivers engaged in "trial and error" healthcare navigation due to limited dementia education and resource knowledge; Gonzalez's men expressed surprise at prediabetes diagnosis despite family history, indicating health literacy deficits prevented translating family health patterns into preventative action. These parallel findings across caregiving and diabetes prevention suggest that health literacy interventions addressing family health management should acknowledge both the complexity of family dynamics (not assuming uniform support) and the structural barriers that limit individual agency.

Healthcare Navigation: Traumatic Brain Injury Program Implementation

Garcia et al. (2025) evaluated implementation of a bilingual, bicultural patient navigation program (BIEN) for Hispanic children with traumatic brain injury across five hospitals in four states. Through interviews with 12 implementation personnel (75% Hispanic, all female, mean age 40), they identified facilitators and barriers using the Consolidated Framework for Implementation Research. Key facilitators included bilingual and literacy-appropriate education materials, bicultural/bilingual navigators who understood cultural norms, and organizational cultures centered on health equity. Significant barriers included program duration limited to 3 months, restrictive policies and funding gaps, and families' hesitancy to apply for assistance due to immigration concerns. This study uniquely examined implementation factors rather than patient outcomes, providing insights into organizational-level requirements for successful culturally adapted health programs.

Population-Level Health Patterns: Obesity and Within-Group Heterogeneity

Sánchez-Johnsen et al. (2025) described baseline characteristics from a community-based participatory research study examining obesity among 199 Latina women (104 Mexican, 95 Puerto Rican) in the Chicagoland area. Mexican women differed significantly from Puerto Rican women on multiple characteristics; young age (38.8 vs 44.0 years, $p=.003$), less likely born in the US mainland, more likely to have less than 9th grade education (17.3% vs 3.2%), higher Spanish language preference (49% vs 81.1% English, $p<.001$), more normal weight (31.7% vs 18.9%), less current smoking (7.7% vs 17.9%), and less insured (50% vs 76.8%). Overall samples BMI was 30.6 and 45% obese. These findings highlight significant within-group heterogeneity among Latina women and demonstrate that health interventions cannot treat "Hispanic" as monolithic, language preferences, acculturation, obesity rates, and health behaviors vary substantially by origin group.

Mental Health and Discrimination During COVID-19

Odame et al. (2025) examined discrimination and mental health among 1,126 Hispanic/Latino adults (55.51% US-born) during COVID-19 through a national online survey (May 2021-January 2022). The ethnically diverse sample included Mexican/Mexican American/Chicano (45.83%), Puerto Rican (15.81%), Cuban (7.02%), Dominican (5.60%), Central American (5.06%), South American (12.17%), and other origins (8.53%). Overall, 38.19% experienced discrimination daily or weekly. US-born individuals reported significantly more discrimination than foreign-born (43.20% vs 31.94%, $p<.001$). Those with anxiety or depression showed 138% higher discrimination risk (RRR=2.38, 95% CL 1.54-3.70); loneliness increased risk 29% per unit (RRR=1.29, 95% CI 1.17-1.42). When stratified by birthplace, anxiety/depression predicted discrimination only among US-born individuals (269% higher

risk, RRR=3.69), while loneliness predicted discrimination for both groups. These findings suggest discrimination experiences differ fundamentally by nativity status, with greater acculturation potentially increasing discrimination exposure.

Substance Use Among Youth: Adverse Experiences and Discrimination

Villamil Grest et al. (2025) analyzed longitudinal data from 1,179 Latino youth (58.95% female, 73.88% second generation) in Los Angeles County (2005-2016), with data collected at mean ages 15.5, 18.6, and 23.9 years. Using latent class analysis, they identified four adverse experience patterns: “Low adversity” (32%), “Psychological abuse & discrimination” (31%), “Psychological, physical abuse & microaggressions" (22%), and “Mainly microaggressions" class (B=.316, p=.04), with no significant differences for marijuana or hard drugs. This suggests the combination of psychological abuse with discrimination is particularly harmful for alcohol use, more so than microaggressions alone, indicating the trauma and discrimination should be addressed simultaneously in prevention efforts. This finding complements Odame et al.’s cross-sectional mental health findings with longitudinal evidence that discrimination’s effects accumulate over developmental periods.

Results and Discussion

This review demonstrates that Hispanic cultural origins shape health behaviors through complex pathways involving familismo, language acculturation, health literacy, cultural beliefs, and discrimination experiences. Several key findings emerged:

First, familismo operates fully as both support and stressor. While family connections can facilitate health behaviors, family demands and unmet expectations for collective support create stress. Interventions must recognize this complexity rather that assuming family involvement is universally beneficial. Notably, familismo appears to function differently across health contexts: In smoking cessation (Correa-Fernández et al.), participants described a spectrum of desired family involvement, with some wanting family support while others found family demands triggered smoking. In dementia caregiving (Estrada et al.), the gap between cultural ideals of collective caregiving and the reality of isolated primary caregivers created profound disappointment and stress. In physical activity intervention (Zlatar et al.), family involvement was valued conceptually by community advisory boards, yet the intervention ultimately focused on individual participants. This variation suggests that familismo’s role is context-dependent, most protective when aligned with individual preferences and actual family capacity, more harmful when expectations exceed reality or when family dynamics create additional burdens rather than sharing them.

Second, health literacy is a powerful mediator. Tailakh et al.'s finding that health literacy explained 62% of variance and predicted 56.1% of variance in preventive behaviors, with education as the strongest predictor (β=.74), suggests improving health literacy through tailored bilingual education should be a priority intervention strategy.

Third, language preferences vary significantly within Hispanic populations. The finding that Mexican women showed much higher Spanish preference than Puerto Rican women demonstrates that interventions cannot assume linguistic homogeneity and must assess individual preferences.

Fourth, birthplace shapes discrimination experiences differently. US-born Hispanic/Latino individuals reported significantly more discrimination than foreign-born counterparts, with mental health symptoms (particularly anxiety/depression) predicting discrimination only among US-born individuals. This suggests different intervention needs by nativity status.

Fifth, acculturation stress operates as a mechanism linking cultural adaptation to health risks, particularly for smoking and substance use. Interventions addressing acculturative stress alongside health behaviors may be more effective than focusing solely on behavior change.

Methodological Strengths and Limitations

The ten studies consisted of diverse methodologies, each contributing unique evidence while presenting distinct limitations. The strongest evidence came from studies combining large samples with rigorous statistical approaches. Odame et al.'s (2025) cross-sectional survey of 1,126 participants used polytomous logistic regression with birthplace stratification, revealing nativity-specific patterns that smaller studies would miss. Villamil Grest et al.'s (2025) longitudinal design following 1,179 youth across three timepoints used latent class analysis to identify distinct adverse experience patterns, demonstrating how longitudinal designs capture developmental processes that cross-sectional studies cannot. Tailakh et al.'s (2025) path analysis with 149 participants provided quantitative evidence of mediating pathways, explaining 62% of health literacy variance and 56.1% of adherence variance, establishing clear directional relationships.

Qualitative studies provided essential depth but faced sample size limitations. Correa-Fernández et al. (n=18), Estrada et al. (n=19), Garcia et al. (n=12), and Gonzalez et al. (n=15) used rigorous frameworks (Social Ecological Model, CFIR, Health Belief Model) to identify themes, yet small samples limit generalizability. However, these studies captured nuances, like the dual nature of familismo and spectrum of desired family involvement, that quantitative approaches might miss. The convergence of findings across multiple small qualitative studies strengthens confidence in these patterns.

Pilot intervention studies (Loya et al., n=21; Zlatar et al., n=10) demonstrated feasibility and preliminary efficacy but lacked control groups and randomization. Both achieved exceptional retention (90.5% and 100%), suggesting cultural adaptation enhances engagement, yet pre-post designs cannot establish causality or rule out regression to the mean, Hawthorne effects, or secular trends.

Critical methodological gaps emerged: only one longitudinal study limits causal inference; zero RCTs means no causal evidence for interventions; small pilot samples (n=10-21) provide insufficient statistical power; geographic concentration limits generalizability; overrepresentation of Mexican/Puerto Rican populations constrains applicability to other Hispanic subgroups; self-report measures introduce bias; and COVID-19 timing may limit generalizability. The methodological diversity serves the field well, qualitative studies generate hypotheses that quantitative studies test at scale, while pilot interventions demonstrate feasibility before costly RCTs. However, the field now needs rigorous experimental designs to establish causality and intervention effectiveness.

Implications for Practice

Effective health interventions for Hispanic populations must begin by recognizing heterogeneity within this diverse group, assessing rather than assuming language preferences and cultural values. Given the significant within-group variation, Mexican women showed much higher Spanish preference than Puerto Rican women, interventions cannot treat "Hispanic" as one large group..

Health literacy emerges as a critical intervention target, particularly through bilingual education, given its powerful mediating role (explaining 62% of health literacy variance and predicting 56.1% of adherence to preventive behaviors). Interventions should address

both individual factors (knowledge, self-efficacy) and structural barriers (work environments, healthcare access, discrimination) simultaneously, recognizing that personal agency alone cannot overcome systemic constraints.

Practitioners must thoughtfully navigate the dual nature of familismo as both resource and stressor, assessing individual family dynamics rather than prescribing uniform family involvement. The variation across contexts, from helpful support in some smoking cessation efforts to isolating burden in dementia caregiving, suggests that interventions should offer flexible family participation options aligned with actual family capacity and individual preferences.

Birthplace and acculturation status should inform mental health intervention design, given that US-born individuals experience significantly more discrimination and show different mental health risk patterns than foreign-born counterparts. Finally, community-engaged approaches using community advisory boards and input from target populations ensure cultural relevance and enhance retention, as demonstrated by the exceptional completion rates (90-100%) in culturally adapted physical activity interventions.

Limitations

As detailed in the Methodological Analysis above, this review's findings are constrained by the predominantly cross-sectional designs (preventing causal inference), small sample sizes in qualitative and pilot studies (limiting statistical power and generalizability), geographic concentration in the Southwest and select urban areas, overrepresentation of Mexican and Puerto Rican populations with minimal inclusion of other Hispanic subgroups, reliance on self-report measures (introducing potential bias), and COVID-19 pandemic timing for several studies (potentially limiting generalizability). Additionally, few studies examined intersectionality or how cultural factors interact with social identities beyond gender and birthplace. These limitations underscore the need for the rigorous experimental and longitudinal designs outlined in Future Directions.

Future Directions

Given the methodological gaps identified, future research should prioritize several specific study designs and populations:

First, randomized controlled trials testing culturally adapted interventions are urgently needed. Building on Loya et al. and Zlatar et al.'s promising feasibility results, adequately powered RCTs (n>200 per arm) should compare culturally adapted interventions to standard care, testing whether cultural adaptation produces superior retention and health outcomes. These trials should examine whether addressing health literacy and cultural factors together produces synergistic effects beyond either component alone. Cluster randomization by clinic or community would allow testing of organizational-level cultural adaptation while minimizing contamination.

Second, longitudinal cohort studies following diverse Hispanic subgroups across decades could illuminate how cultural factors and health behaviors change across generations and life course. Following first-, second-, and third-generation immigrants from adolescence through older adulthood would clarify whether the observed paradoxical acculturation effects (greater discrimination among US-born) persist, intensify, or diminish over time. Such studies should oversample underrepresented groups (Cuban, Dominican, Central American, South American) to enable subgroup comparisons and should include biomarkers alongside self-report to reduce measurement bias.

Third, mediation and moderation studies using structural equation modeling or causal mediation analysis could test specific pathways linking cultural factors to health. For example, does health literacy mediate the relationship between language preference and preventive behaviors? Does familismo moderate the effect of acculturation stress on substance use? Such studies require samples of n>500 to detect modest mediation/moderation effects with adequate power and should measure constructs repeatedly to establish temporal precedence.

Fourth, mixed-methods studies combining quantitative measurement of health outcomes with qualitative exploration of mechanisms would leverage the strengths of both approaches. For instance, RCTs of culturally adapted interventions could embed qualitative interviews with high and low responders to understand why cultural adaptation works for some but not others, informing intervention refinement.

Fifth, multi-level intervention trials testing simultaneous individual, family, community, and policy-level changes could address the multiple levels at which cultural factors operate. For example, combining patient-level health literacy education with provider cultural competency training and clinic-level bilingual staff hiring would test whether multi-level approaches produce effects beyond individual-level intervention alone.

Sixth, intersectionality research examining how cultural factors interact with gender, sexual orientation, disability status, and socioeconomic position remains critically needed. Latent class analysis or finite mixture modeling could identify subgroups experiencing distinct configurations of cultural resources and stressors, enabling more targeted interventions.
Finally, studies should examine protective cultural factors and resilience alongside risks. The field has documented how familismo creates stress but has insufficiently explored when and how it confers protection. Positive psychology approaches identifying cultural strengths could inform asset-based rather than deficit-based interventions.

Specific high-priority study designs include: (1) a multi-site RCT (n=400) testing culturally adapted diabetes prevention in Cuban, Dominican, and Central American populations currently underrepresented in research; (2) a 20-year cohort study (n=2,000) following Hispanic youth across first through third generation to map acculturation trajectories and health outcomes; (3) an implementation science hybrid trial testing real-world delivery of culturally adapted physical activity interventions through Federally Qualified Health Centers while examining implementation barriers and facilitators; and (4) ecological momentary assessment studies capturing real-time experiences of discrimination, acculturative stress, and health behaviors to clarify temporal dynamics that retrospective surveys obscure.

Conclusion

This review of recent empirical research demonstrates that Hispanic cultural origins shape health behaviors through complex, multifaceted pathways. Effective health interventions must recognize within-group heterogeneity, prioritize health literacy as a key mediator, address both individual and structural factors, and thoughtfully navigate the dual nature of cultural factors as both protective resources and potential stressors. The strong evidence for health literacy as a mediator suggests that improving health literacy through tailored bilingual education should be a priority intervention strategy.

Among the methodologies reviewed, longitudinal designs with large samples (Villamil Grest et al., n=1,179 across three timepoints; Odame et al., n=1,126) and path analysis with adequate statistical power (Tailakh et al., n=149) produce the best quality evidence in terms of reproducibility and error rates. These studies provide clear effect sizes, confidence intervals, and statistical controls that enable replication. In contrast, small qualitative studies (n=10-19) and pilot interventions without control groups, while providing valuable preliminary insights, have lower reproducibility due to small samples and lack of randomization. The critical gap is the complete absence of randomized controlled trials, no study in this review establishes causal relationships or intervention efficacy through experimental design. Additionally, underrepresentation of Cuban, Dominican, Central American, and South American populations (appearing in only 1-2 studies each) limits generalizability across Hispanic subgroups. Given these quality assessments and gaps, the next research steps should prioritize: (1) adequately powered RCTs (n>200 per arm) testing culturally adapted interventions against standard care to establish causality; (2) longitudinal cohorts (n>2,000) following diverse Hispanic subgroups across generations to clarify acculturation trajectories; and (3) replication studies confirming key findings (particularly health literacy mediation and familismo's dual nature) across different Hispanic populations and geographic regions.

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Claude was used for grammar and structure check/cleaning up, MyBib was used for keeping track of my sources and helping format my citations, Google docs was used to draft this first draft. Canva was used to create figures 1 and 2, Jupiter Notebook (python) was used to create figures 3 and 4.